



SUMMARY

Mark Jenkins has lived on Alabama's death row for more than 35 years and is described by correctional officers as a "model inmate" who has consistently exhibited kindness, dependability, and consideration for others for decades. His childhood was marked by some of the most severe abuse I have ever assessed in my 20 years of working in interpersonal violence treatment and evaluation, and he developed Complex Post-Traumatic Stress Disorder due to those childhood exposures. Mark exhibited serious learning problems from a young age due to intellectual disability. His mother, and the man he was raised to believe was his father, rejected Mark and subjected him to extensive severe physical, psychological, and emotional abuse on a daily basis, literally leaving him to eat trash while they excluded him from family meals, locking him away for long periods of time, and forcing him to engage in degrading and humiliating punishments when he expressed any distress about his abuse. He was sexually abused in early childhood when he was raped by his grandfather. The type and severity of abuse Mark endured is highly unusual even compared to other seriously traumatized individuals. He did not have a history of significant aggression during childhood or adolescence before the instant offense in 1989, despite the severe abuse and neglect he endured, and instead is described by family members and friends as a sweet and caring child who was fond of animals and desperate to be loved and accepted by his parents.

At age 58, given his criminal history and institutional record, Mr. Jenkins is at very low risk of general recidivism or violent recidivism. He consistently expressed a degree of remorse and guilt that is highly unusual in my experience, and he has sought to build and maintain positive, prosocial relationships with healthy individuals outside of the prison environment as well as respectful relationships with correctional officers at Holman. I cannot recall another individual whom I've evaluated for a capital or other serious felony case who had two, let alone more than 60, correctional officers willing to write letters on their behalf. Mark continues to present with intense feelings of remorse and guilt, and cognitive limitations, but he has grown out of the adolescent developmental factors that, in combination with his severe trauma history and acute intoxication at the time of the offense, are explanatory with respect to the offense conduct, which was the murder of Tammy Hogeland in 1989. He would be at low risk to violently reoffend whether he spent the rest of his life in prison or was in the community. This assessment that Mr. Jenkins presents a low risk for violence is not only my opinion, but also the opinion of the correctional officers who saw him on a near-daily basis for decades, and it is congruent with the result of a formal risk assessment conducted by a prison violence risk expert. Mark's case is unique for his severe trauma victimization history, his lack of serious violent conduct outside of the instant offense, and his low risk for violence in the future regardless of setting.

Date of report: September 4, 2026

Identifying Information:

Re: Mark Jenkins
DOB: September 13, 1967 (58 years old)
Age at offense: 21 years old

Referral Questions: At your request, I have completed a psychological evaluation of your client, Mark Jenkins. This letter provides a report of my evaluation procedures and findings, specifically with regard to psychological history and psychological factors relevant to the offense conduct and violence risk, and his current psychological status.

SOURCES OF INFORMATION

I interviewed Mark Jenkins at Holman Prison on December 20, 2023, and July 3, 2025. In addition to interviews, I reviewed copies of the following records in the preparation of this report:

1. Excerpts of Mr. Jenkins's educational records from childhood.
2. Birth record.
3. Childhood mental health records.
4. Neuropsychological evaluation report authored by Robert H. Ouaou, Ph.D., dated May 3, 2023.
5. Taylor Hardin Secure Medical Facility records.
6. Alabama Department of Corrections records related to medication, medical treatment, medical requests, and progress notes.
7. Records related to Mr. Jenkins's encounters with juvenile justice authorities.
8. Presentence Investigation Report dated March 18, 1991.
9. Records related to Mr. Jenkins's mother, Donna Jo Jenkins.
10. Records related to Pamela Montes (sibling), Steven Jenkins, Jr. (sibling).
11. Transcripts of Steven Jenkins, Jr.'s testimony, and Tammy Shidler's (cousin) testimony.
12. Legal records and a news report related to William Mark Jenkins, Jr.'s criminal cases in San Bernardino County Superior Court.
13. Declarations/affidavits from over 30 correctional officers from Holman Prison speaking to Mr. Jenkins's conduct and character during his incarceration on death row.
14. Declarations/affidavits from Jerry Tindell, Kimberly Fawcett, Jennifer Montes, Vickie Thornburg, Charles Blanton, Vicki Blanton, and Shari Seale. These documents relate witness recollections of Mr. Jenkins's abuse and neglect in childhood, among other topics.

15. Report of Jon Sorensen, Ph.D., regarding violence risk assessment in general population results for Mr. Jenkins.

REVIEW OF PSYCHOLOGICALLY RELEVANT FACTORS FOR MARK JENKINS

Mark's Trauma History is Unique in the Severity, Variety, and Pervasiveness of the Abuse He Experienced Starting in Infancy

Mark Jenkins was born in Long Beach, California, in 1967. He had two brothers, William and Steven, Jr., and a sister named Pamela. Mark had a different father than his siblings, but he was reportedly not informed that Steven Jenkins, Sr. was not his biological father until Mark was about 12 years old. Mark had a different appearance than his siblings, specifically he had darker skin, and he recalled that he was ridiculed and treated differently from his brothers and sister. He was reportedly the product of a brief relationship between his mother, Donna Jenkins, and a sailor with the last name of Mendoza. Mark was conceived while his stepfather (Steve Jenkins, Sr.) was in prison. A 1981 report stated, “[Mark’s father] has a lot of bitterness towards his son because of his being born to his wife while he was in prison.”

It is unambiguous that Mark was severely abused within his family. Mark was raped by his grandfather when he was four years old. That night, after the abuse, Mark urinated in the sleeping bag he was sleeping in next to his father.¹ Mark’s grandfather was not only abusive to Mark; he abused his daughters and was physically violent and assaultive toward his children. He was reportedly a violent alcoholic. Dr. Lisak, one of the evaluating psychologists, observed, “There is not only the impact of the trauma of the sexual abuse itself, but the events that in retrospect were clearly set in motion by Mark’s symptomatic reaction to this sexual abuse.” For example, Mark regressed and began bedwetting again after his grandfather raped him. Dr. Lisak indicated that, according to his evaluation, Mark was also locked in his room, alone, on many occasions. His siblings were reportedly recruited to participate in Mark’s abuse, which both made the other children complicit in child abuse and also eliminated any chance of Mark being able to seek help and support from his siblings. He was beaten with metal rods, a wooden paddle, boards, and closed fists, and physical and emotional abuse were “routine.”

Mark usually coped with abuse by trying to escape (run away) or hide, and the records and history indicate that **Mark did not respond with violence, even in self-defense, when he was abused**. Mark recounted one of his early experiences with running away from home when I interviewed him at

¹ Loss of bowel and bladder control is a relatively common (but not necessarily well-known) trauma response, often related to the neurobiological processes that characterize human responses to severe threats (i.e., the fight or flight response). Many sexually abused children exhibit a regression in toilet training after the onset of abuse.

Holman Prison in December of 2023: “I started running away at 12. I was free. The first night I ran away from home, I spent behind a liquor store. There was a brick wall, it went all the way down the middle of it. I sat down and went to sleep. I didn’t wet myself. I felt safe. I don’t wet the bed at all unless I get scared.” **It is noteworthy that Mark referred to sleeping behind a liquor store, outdoors, at age 12, as a time when he felt safe.** His account underscores how horrific his home life was, for sleeping outdoors behind a liquor store to feel like relief and safety.

At home, Mark would try to remain with the animals, because he “felt safe around animals. They were the best thing that ever happened to me.” Mark specifically remarked on having a soft spot for animals who were abused and neglected, and his desire to care for them. This is consistent with reports, including declarations from correctional officers at Holman and Jerry Tindell, that speak to Mark’s longstanding attachment to and fondness for animals.

Mark blamed himself for the abuse visited on him by his family—the same people who should have loved and protected him. He was cognitively impaired and had no one to counteract the family messages that he was unworthy, unlovable, and unwanted. He recalled,

My dad said I’d never graduate high school, he said I’d be in prison. And I felt like I deserved it. I felt like because I was wetting the bed, that I deserved it. When it was spanking, it was ok, but by the time they were done they hit you all over your face, head, legs. Mom was like that, she’d start hitting you and then she was hitting you everywhere... Mother would hit me with those hot wheel tracks. Mom used everything.

Records from a children’s continuing care program in 1982 note obvious neglect of Mark, and overt rejection and abandonment by his mother and stepfather:

There is general agreement from friends, grandmother, even a local detective, that Mark is ignored and rejected by his family, that he has been reared in conditions that at least border on neglect and abuse....His parents have not visited him while he has been in placement, nor did they visit him in his lengthy stay at Juvenile Hall, not even on his birthday. They have not been willing to participate in any family counseling. He has evidently had quite a rejecting family. This past month, during the Easter break, Mark had been looking forward to going home during this time and at the last moment his family refused to have him return home for Easter week. It is unclear as to what the discharge plans will be since his parents have been so rejecting of him...

[Mark] has done quite well in the program. It is only when he has heard or not heard from his family that he begins to regress.

Around this time, Mark was diagnosed with mood disorder as well as a mixed specific developmental disorder. He was referred for a Special Education program at school, and "in spite of his academic failures, the boy has expressed liking for school and he intends to graduate." When Mark was asked about the onset of rule-breaking behaviors around age 12 or 13, he reportedly responded that "he feels he has many problems with his parents which he does not understand and as a result he would rather run from them than confront them."

The residential placement records noted that Mark's mother did not follow up with recommended treatment in the community for Mark. Records from Mark's childhood noted that he was seen by a psychologist, Martha Berkey, who generated a report that was referenced in other records. The psychologist described Mark as follows: "The underlying psychodynamics reflected in testing are similar to those ascribed to victims of early emotional and physical child abuse. Mark is a very confused, young, adolescent who experiences strong emotional pulls between love and hate." Mark reportedly stated that he believed **he was unloved and unwanted at home, and that the only way he could avoid staying with his family was to be placed outside the home by the Probation department.**

A November 1981 report stated, "[Mark's] insight into his present predicament as far as not being wanted at his home and being cared for is somewhat appropriate. However his judgment in regard to relieving the situation is definitely impaired....It is apparent to the interviewer that Mark is actively seeking support and love from whatever source he can attain it from." Mark was so desperate to be loved that he sought care from unrelated adults in out-of-home placement settings, and was so hurt and afraid of his family that he would rather be in a probation placement for disturbed youth than be at home. Mark was placed at Ahwanhee Hills School, with the rationale that the school would be "a suitable setting for the minor primarily because it is a large distance away from parental contact and at the present time it is deemed that this is necessary so that minimal contact is maintained with the family." **In about 14 years of doing forensic psychological evaluations, and more than 20 years of working with/treating abused and neglected children, I have never seen a comparably strong recommendation that a juvenile be placed far from their parents for their own well-being.**

Mark's account of his childhood abuse was corroborated by many individuals who witnessed the abuse firsthand, and some of these individuals found merely witnessing Mark's abuse to be traumatic for them. Jennifer Montes, Mark's niece (his sister Pamela's daughter), described Pamela's severe mental illness and chronic suicidality, as well as the chaos, deprivation, and serious neglect in the Jenkins household (where Ms. Montes had to live at times during her childhood, after Mark had already left home). Ms. Montes recalled that her mother accused Steve Jenkins, Sr., of sexually abusing her (Pamela), stating that Pamela's disclosure "caused a big fight in the family."

Ms. Montes provided her recollections of the severe drug and alcohol abuse, methamphetamine manufacturing, violence, and neglect in the Jenkins home. Ms. Montes stated,

I still can't believe that we were never taken away from Steve and Donna and placed in foster care. It amazes me. We should have been removed from that home...Living with Steve and Donna was like living in hell. I had it pretty bad growing up, but Mark had it much, much worse than I did. Mark's entire life was shaped by the terrible ways he suffered at Donna and Steve's hands.

Vickie Thornberg, Mark's paternal aunt, experienced Mark's grandfather's violence firsthand, and witnessed Mark's maltreatment by his parents, writing about both in her declaration. She described the severe and unpredictable physical, sexual, and emotional abuse inflicted on herself and her siblings by Mark's grandfather, Robert Jenkins. Robert Jenkins raped Vickie from when she was 12 years old until she told her mother, but he did not stop sexually abusing children—he merely moved on to Vickie's younger sister. Vickie remembered when Mark was a small child who "loved to sit on [Vickie Thornberg's] lap. It seemed like he was looking for someone to care for him. He was starved for love." Vickie corroborated the severe abuse and neglect that Mark experienced from both of his parents. She stated in her declaration, "I don't think Mark received kindness from anybody when he was little, not from anyone he lived with, at least. He was so mistreated, it's hard to think about it without crying, still."

Kimberly Fawcett, Mark's cousin, lived in the same house with the Jenkins at times during her childhood. She confirmed Steve Jenkins, Jr.'s abusive mistreatment of Mark as well as his sexually inappropriate behavior toward young girls in the family. She recalled witnessing Steve Jenkins beat Mark with "anything that was handy," humiliate Mark, and verbally abuse Mark. Kimberly was distressed, even at the time of her declaration in 2024, about the failure of adults to intervene to protect Mark, even though they knew what was happening to him:

I was just a kid, and I still knew something was wrong when I heard Mark crying in pain. All of the adults knew what was going on. They saw how cruel Steve and Donna were to Mark. I don't know why the adults did not do anything about it, and **it is really hard for me to think about how the adults I loved just looked the other way.**

People outside of the Jenkins family also corroborated Mark's abuse and other maltreatment. Jerry Tindell knew Steve Jenkins, Sr. and the Jenkins family in the 1970s and 1980s. Mr. Tindell recalled that Steve Jenkins, Sr. spoke with pride about how he disciplined Mark by forcing Mark to eat his own feces, noting that it was Steve's way of "taking control and showing Mark who was boss." Mr. Tindell described Steve's abuse of Mark as "relentless," and stated that he never heard Steve make any positive statements about or to Mark.

Both of the experts who testified at Mark's post-conviction proceeding, including the government's expert, Dr. Kirkland, described the abuse that Mark experienced and the likelihood that he was

profoundly affected by his early life trauma and the chronic abuse inflicted by his family members. For example, Dr. Lisak noted that traumatized children “become behavior problems in the classroom because they are not sitting still and so forth to get the teacher’s attention...they are spending their time not learning about the world, but rather focusing on the adult or whoever is hitting them or whatever. There are pervasive impacts on their cognitive development.” Both experts indicated that Mark was physically abused, severely emotionally abused, sexually abused, and medically neglected by his parents, who both had chronic substance use problems. The abuse reportedly included severe humiliation, such as forcing Mark to eat feces and live like an animal, separately from the rest of the family. Much of the abuse appeared to be focused on a reaction to Mark becoming incontinent of his bowels or bladder, and so abuse was used to punish Mark for reacting to the sexual abuse he had already endured. Of note, Dr. Lisak is a well-regarded trauma psychology expert.

Even at the time of our interviews, Mark endeavored to find a way to see his parents in some positive light. For example, he recalled, “I didn’t know if my father loved me. I didn’t feel like they loved me. I had to work all the time. Scraping gaskets and heads off blocks at seven years old, climbing under a car to work on a transmission. He taught me a lot about work, I am giving him that praise today.”

Mark ultimately escaped his family home and lived on the streets as a teenager, without stability in housing, location, employment, education, access to treatment, or social support. He was a traumatized, and—as discussed below—disabled child who attempted to care for and protect himself but was unable to do so. He blamed and hated himself for his emotional and cognitive problems, and for being abused and exploited, and he desperately longed for connection, acceptance, and secure attachment. From about 1983 onward, Mark was part of “homeless drug culture” in California, meaning that he was a teenager being socialized to a culture that was (a) the only place he was accepted, (b) lacking in lifestyle stability or prosocial role models, and (c) modeling substance use as a way to cope with the world and survive.

Mark Has Lived with an Unaccommodated, Untreated Developmental Disability Since Childhood

Mark was born prematurely, and he remained in the hospital for weeks, according to records. He was likely exposed to alcohol and drugs in utero.² His mother and father were listed as separated on some early medical forms.³ His weight was documented as five pounds one ounce, which is below the second percentile for male infant birthweight, meaning that less than 2% of other male infants weighed less. Initially records showed that he was in good condition, but records noted that he was not growing at the rate expected.

² See page 27 of Dr. Ouaou’s report.

³ See MJ-000095.

Educational records noted that Mark made a strong effort at learning academic skills, but he lagged behind other children and showed signs of significant cognitive impairments. By fifth grade, he already was significantly below grade level, and his educators expressed concern:

Math Level: Below ☒ At ☐ Above ☐
Reading Level: Below ☒ At ☐ Above ☐
Language Arts Level: Below ☒ At ☐ Above ☐
Mark is new to the school about mid-quarter. He is a pleasant young man who tries hard. His grades are low now because of his lack of fundamental skills in reading, math, and language.
Date: 10- -77 Parent: _____

[Handwritten text reads: "Mark is new to the school about mid-quarter. He is a pleasant young man who tries hard. His grades are low now because of his lack of fundamental skills in reading, math, and language."]

Mark's educational records showed that he was making an effort, but was not able to perform at grade level:

FIRST QUARTER CONFERENCE:
Grades are based on performance at:
Reading: Below ☐ At ☐ Above ☐ Grade Level
Language Arts: Below ☐ At ☐ Above ☐ Grade Level
Math: Below ☐ At ☐ Above ☐ Grade Level
COMMENTS: Mark is working very hard. He always tries to get his work done on time but most of the time finds it very difficult. Mom so well pleased with his effort and his self control. I hope Teacher: He can get some extra help.

SECOND QUARTER REPORT CARD:
Grades are based on performance at:
Reading: Below ☐ At ☐ Above ☐ Grade Level
Language Arts: Below ☐ At ☐ Above ☐ Grade Level
Math: Below ☐ At ☐ Above ☐ Grade Level
COMMENTS: Request a conf. with mother please.
Teacher: _____ Parent: _____

THIRD QUARTER CONFERENCE:
Grades are based on performance at:
Reading: Below ☐ At ☐ Above ☐ Grade Level
Language Arts: Below ☐ At ☐ Above ☐ Grade Level
Math: Below ☐ At ☐ Above ☐ Grade Level
COMMENTS: The work is too hard for Mark.
Teacher: _____ Parent: _____

FOURTH QUARTER REPORT CARD:
Grades are based on performance at:
Reading: Below ☐ At ☐ Above ☐ Grade Level
Language Arts: Below ☐ At ☐ Above ☐ Grade Level
Math: Below ☐ At ☐ Above ☐ Grade Level
COMMENTS: Mark isn't fair. He has worked hard this year and made some growth, but 5th grad. is too far above the level he is working. He has about 2 or 3 grad levels to catch-up. At his own level he is doing well. Mark
Teacher: _____ Assigned to Grade _____

In fifth grade, Mark's achievement scores were nearly all still at the third grade level. In sixth grade, testing documented in his school records showed that he was at the second grade level for vocabulary (at the 3rd percentile), and the second grade level for reading comprehension (5th percentile). In math-related areas, he functioned at the first and second grade level.⁴ By sixth grade, Mark began to exhibit more outward indications of poor mental health functioning, including restlessness, irritability, and "profanity," but educators also noted that he was markedly immature.

⁴ See MJ-000038.

One educational record noted,

Frequent moves, changes in schools and family instability have made it difficult for Mark to achieve success in academic subjects. Low ability coupled with a specific learning disability add to his problem so that, at times, behavior is a reflection of inferiority feelings, rejection by peers and general lack of success in school life.⁵

Mark began to skip school and run away from home around 6th grade. Records indicated that he was fearful of seeking help and was rejected and abused by his family members. A note from 1979 stated that Mark (at age 11) had "marked gingivitis," and noted that Mark said, "he fears getting into trouble for seeking care in the nurse's office." These records also noted that Mark was "frequently unkempt." He repeated 7th and 9th grades.

Mark was diagnosed with mood disorder as well as mixed specific developmental disorder, and referred for Special Education at school. *Mixed specific developmental disorder* referred to a global set of delays, spanning motor, language, and social development, very commonly associated with an underlying intellectual (or other serious cognitive) disability.

Like many people with developmental disabilities, Mark tried to find a way to cope with, and conceal, his vulnerabilities. Lonnie Seale's wife, Shari Seale, described helping Mark with basic tasks at home, saying that she "thought Mark had some kind of mental disability." Her assessment was in part because he was "slow," and required concrete step-by-step instructions even for straightforward tasks like retrieving a can of dog food. He was reportedly exploited by his employers and "had no idea how to manage his money." Shari was willing to help Mark, and she described him as "kind and loving but also timid, withdrawn, and lonely."⁶

Later testing conducted by Dr. Kirkland (the government expert at post-conviction proceedings) and Dr. Ouaou produced Full Scale IQ scores within the range required for a diagnosis of Intellectual Developmental Disorder (IDD). Most recently, Dr. Ouaou offered this diagnosis formally in his 2023 report, based on a comprehensive review of records, interviews, and neuropsychological testing. Intellectual Developmental Disorder is also referred to as Intellectual Disability (ID). ID is a type of cognitive impairment that falls under the umbrella of developmental disabilities.

Affidavits and declarations from correctional officers also attested to Mark's limited intellectual ability. For example, Michael Branch stated, "Mark did not appear very smart to me. Just from general conversation, he seemed a little slow." Eddie Dunsford's declaration stated, "Mark was very slow, mentally. I'm not a psychologist but I think he had some kind of learning disability or problems...It

⁵ See MJ-000042.

⁶ See MJ-003042 through MJ-003046.

also seemed that he did not understand everything that was going on. He never got mad. He was extremely gullible.”

Mark’s Trauma-Related Injuries Compounded His Intellectual Disability

Mark’s cognitive limitations arising from his ID impaired his ability to comprehend his circumstances, advocate for himself, and problem-solve his situation effectively. Much of his abuse also occurred when he was very young, even before he was able to speak, and consequently abuse and maltreatment victimization were normalized for him by the time he was a small child. This likely contributed to the harm that was caused by his trauma symptoms.

Young children can be profoundly injured by even a single traumatic event, in part because they are so small and helpless. For an adult, this would be comparable to being surrounded by giants at nearly all times, subject to their whims and impulses and lacking any rights or recourse, dependent on them for food and shelter, and unable to predict when they will violate or otherwise harm you. When adults around you decide to harm you, you are helpless to stop them for the most part if you are a child. This contributes to the terror and self-blame that children experience in response to abuse victimization, and Mark was no different. He blamed himself for the abuse and neglect, believing he must have deserved it. Remarkably, prison appears to have been the first place Mark lived where he had stability. His traumatic experiences are striking, even compared to the dozens of other individuals whom I’ve evaluated in capital proceedings before and since meeting Mark.

For a child with developmental disabilities, the types of abuse Mark experienced would be even more perplexing and disorienting. Children with developmental disabilities often evoke strong punitive reactions from adults who believe the child should be able to do things (e.g., toileting, reading, understanding verbal directions, doing chores) that the child cannot do because of their disability. The adults interpret the child’s inability as weakness or oppositionality/disobedience, and punish the child abusively. Mark was punished not only for his mother’s infidelity, and his racial background, but also for his disabilities and the trauma symptoms he ultimately exhibited due to abuse and neglect (e.g., bedwetting). Children and youth with developmental disabilities and trauma histories usually do not have the tools they need to cope with the injuries they have sustained, and to keep themselves safe. They often idealize their perpetrators and other relationships, putting people on a pedestal and devaluing their own needs and wants, to maintain relationships. When these individuals feel betrayed, they usually either blame themselves, pretend the betrayal did not occur, or react with splitting. Splitting is a marked and rapid change in their assessment of the other person, seeing the formerly valued person as a threat and/or a monster instead of being able to adopt a more nuanced and reality-based understanding of the relationship and person. Mark has demonstrated difficulties throughout his life in terms of understanding nuance and being able to live with ambivalence about the family members who harmed him.

Records from Mark's time in Taylor Hardin in 1990 characterized him as depressed, "limited in intelligence," and childlike, consistent with his developmental disability. He impressed as overwhelmed by his circumstances and reportedly had difficulty comprehending questions and instructions. Unsurprisingly, he has continued to exhibit gullibility and a vulnerability to exploitation by individuals in prison who are more predatory, manipulative, and intellectually capable than he is. Mark described feeling fearful and anxious at times, because he knew people would try to trick and exploit him, but felt unable to assert himself or successfully recognize when others had bad intentions toward him. In other words, Mark has continued to try to cope with his developmental disability while incarcerated, with mixed results.

The definition of IDD has changed over the years, from the Diagnostic and Statistical Manual, Second Edition that was in effect when Mark was a child through the current version (Diagnostic and Statistical Manual, Fifth Edition Text Revision), published in 2022. Based on my review of the information, including Dr. Ouau's report, **if Mark were a child in school today he would likely be identified as a child with both learning disabilities and ID by educators based on current special education practices and diagnostic criteria for intellectual/developmental disabilities.** The significance of his ID is that it increased his risk of victimization, decreased the likelihood that he would be able to advocate for himself or problem-solve his circumstances as a child, and likely contributed to his social-emotional immaturity and delayed psychological maturation process. Having Mark's ID diagnosed would not have changed the fact that Mark's family members abused him. However, he would have had access to more help, support, and ultimately resources (after the Home and Community-Based Services waivers went into effect around 1981). These waiver services could have provided housing, supervision, educational assistance, transportation, medical care, and mental health care for Mark, promoting both his recovery and community safety.

Mark Rejects Violent Values, Feels Unusually Intense Guilt & Remorse, & Strives to be Different

Mark has retained an intense and childlike desire to feel connected to his biological father, and distress that he was unloved by his stepfather and mother. He asked me during our interview if I might be able to find information about his biological father, and records indicated that Mark has been asking for help finding his father for at least 35 years. At the same time, Mark's desire to be close to and find out more about his biological father co-exists with his feelings of despair and unworthiness when he thinks about his mother and Steve Jenkins, Sr. Mark's desire to find his birth father is partly attributable to his desire to see himself as different from his mother and stepfather, and to be able to see himself in a positive and prosocial light. For example, after having been made fun of for his skin color and different appearance (compared to his siblings), Mark reportedly saw what he believed to be a picture of his biological father. In the picture, the man had wavy hair, and Mark recalled trying to comb his hair to effect a similar appearance. Mark's desire to change himself, in order to be acceptable and worthy of love, has persisted despite the more general pattern of social-emotional maturation.

In contrast to his desire to find a feeling of connection to his biological father, Mark has also expressed a desire to avoid repeating the behavior that his stepfather, Steve Jenkins, Sr., exhibited toward his children. At the time of our interviews, although Mark clearly still wished that his stepfather had been more caring and kind, he also expressed disgust and distress at his stepfather's conduct toward other people, particularly his physical and emotional cruelty. Mark said that part of his acute guilt about the crime against Ms. Hogeland was that he had engaged in violence and in that respect followed Steve Jenkins, Sr.'s example. Mark said that, although one of his most strongly held values was the need to reject violence and to try his best to be kind and helpful to others, he knew that he could never undo the loss of Ms. Hogeland's life, and therefore might never be able to truly extract himself from his father and grandfather's cycle of violence. When I asked Mark to tell me the difference between his presentation now and how he was 30 years ago, he replied,

[I] finally stood up and said, 'I need to take responsibility for my actions as a man.' I tried to blame everyone and everybody for everything that has happened to me. [It] took a long time, to come to grips, that I shamed myself. I hurt another family. I took the life of a beautiful person...You can't pay for it. You can't fix it, you better own it.... I took a life and I cannot give it back. I would give my life to give it back.

The letters from the correctional officers at Holman provide repeated and remarkable support for Mark and affirm the evident sincerity of his remorse and feelings of guilt. I observed that the officers commented on Mark's character and repentance in their declarations; this is highly unusual in my experience. To the extent that I have encountered letters from any law enforcement-type personnel, including correctional officers, writing based on their professional experience with an incarcerated person, these letters have generally focused on work performance, lack of disciplinary infractions, and the reliability/dependability of the individual. In Mark's case, the officers offer substantially more commentary about the nature of their interactions with Mark, his history and characteristics, and their perspective that he is at low risk of future violence. Specifically, I noted the following:

- The vast majority of correctional officers who submitted affidavits and declarations had 20 years or more of service within the Alabama Department of Corrections, so these officers had years of experience and hundreds of individuals against whom they could compare Mark Jenkins.
- Many officers described Mark as fundamentally and characterologically distinct from other individuals on death row in terms of his passivity, absence of anger/hostility, compliance, and childlike nature.
- Most of the officers noted that they had never signed similar statements about any other incarcerated person, but were making an exception for Mark because he is unique.

- Mark was consistently described as calm, respectful of authority figures, reliable, rule-adherent, gentle, avoidant of conflict/violence, and remorseful.
- None of the letters referenced any reservations about Mark's ability to live in general population for the remainder of his life without harming others. None described Mark ever expressing anger, hostility, or aggression toward anyone.

Mark has expressed intense remorse, shame, and guilt related to the death of Tammy Hogeland. He made numerous references to feelings of guilt, shame, and profound sadness regarding his crime, and often said he struggled to receive praise from others about good conduct or character because he felt he could never reconcile this praise with the instant offense conduct.

The letters from correctional officers also noted Mark's expressions of remorse and his commitment to rehabilitation. For example, Mitchell Blair (who worked on death row for 15 years) noted, "When you interact with people as much as we interacted with inmates, you get a sense of how they're thinking and feeling. **Mark really regretted what he had done. He felt deep remorse for his crime...There was a mighty powerful change in him from the person who committed the crime.**" Freddie Howard, a 29-year veteran of the Alabama Department of Corrections, observed, "[Mark] expressed remorse and showed that he wanted to make himself a better man than he was when he committed his crime. He had a good temper and was never violent, disruptive, or anything like that."

Mark Has Matured into Middle Adulthood and Aged Out of the Developmental Psychology Factors that Likely Played a Significant Role in the Offense

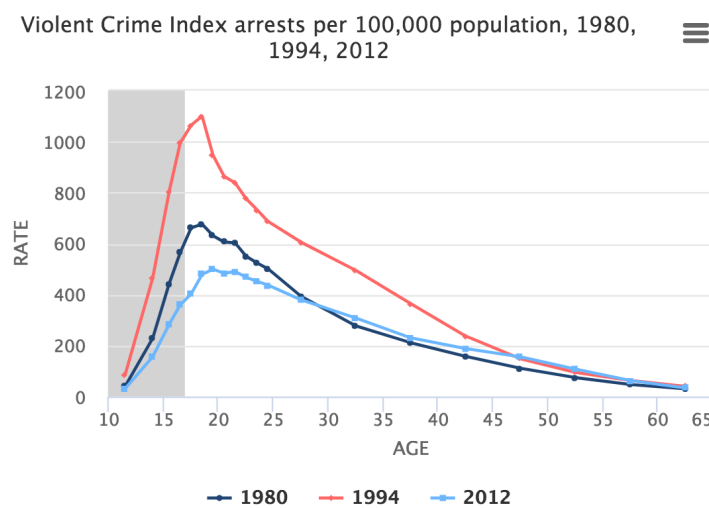
Consistent with the observations of the correctional officers who submitted declarations on behalf of Mark, the research literature is supportive of the view that Mark was likely immature, with incomplete brain development, at the time of the offense, even though he was legally past the age of majority in Alabama at that time.

It is now firmly established that adolescence is a unique developmental time period in the human lifespan. A number of changes in brain structure and function occur during this time, and the rate of change is so rapid in adolescence that it is surpassed only by the fetal and infancy stages of human development. Some of the most prominent and relevant changes include (a) shifts in proportion of gray to white brain matter, (b) rapid and early development of the brain regions associated with reward processing, and (c) lag of the prefrontal cortex, in terms of timing, behind other brain systems.

The consequence of the adolescent developmental psychology process is that youth between the ages of about 12 and 24 are highly sensitive to environmental factors, like the presence of peers. Adolescents are impulsive, with poor judgment, and as a consequence we see increases in risky

behavior and accidental injuries during the same time period when they are most sensitive to situational/environmental factors and have not yet completed development of the prefrontal cortex (which is the last brain region to finish development, and is responsible for most of our decision-making and reasoning).

For adolescents, the prefrontal cortex maturation lags behind other developmental processes, and when that brain region comes online in the 20s we see fewer risky and illegal behaviors. See, for example, the violent crime arrest rates by age,⁷ below:



Note: The Violent Crime Index includes the offenses of murder and nonnegligent manslaughter, forcible rape, robbery, and aggravated assault.

In Mark's case, the adolescent developmental peer influence factor is significant primarily with respect to Mark's social influence when he was ages 13 to about 21. He showed significantly better functioning and stability when he was living with Lonnie Seale and his family, and much worse functioning when he was unhoused, had no one to help him, and was drifting among heavy drug users on the street. A critical lesson from the adolescent developmental psychology research literature is that an episode of behavior during adolescence often does not reflect a stable trait of the individual, and consequently most authoritative texts on violence risk assessment encourage evaluators to recognize adolescence as a time of rapid change and adolescent evaluatees as "moving targets" with many opportunities for effective intervention.

Adolescence is a time when situational factors, like peer influence, substance use, and emotional stress, are stronger determinants for behavior than individual characteristics like personality. In other words, for teenagers up through the early to mid 20s, situational factors are often more important than individual child characteristics. In adults, situational factors still matter, but to a much less

⁷ OJJDP Statistical Briefing Book. Available: <https://www.ojjdp.gov/ojstatbb/crime/qa05301.asp?qaDate=2012>.

marked degree compared to adolescents. So, **Mark currently is not vulnerable to those peer influence factors in the same way, because he has outgrown this developmental psychology adolescent phase.**

Trauma-Related Developmental Considerations: Traumatology research, including neuroimaging studies, have revealed that **abuse can modify the physiological makeup of the brain in ways that hinder brain development and undoubtedly play a role in the emotional, cognitive, and behavioral sequelae of childhood trauma.** For example, the hippocampus plays a critical role in learning and memory, and high levels of the stress hormone, cortisol, may adversely affect the hippocampus. Studies have shown anatomical difference in limbic structures (the amygdala and the hippocampus) in the brains of maltreated children versus non-maltreated children. This research was alluded to in Jennifer Montes's declaration, when she describes the "brain scans of children who were neglected and abused next to brain scans of children who were cared for and loved"⁸ in the context of her work as a psychiatric technician supporting individuals with serious mental health problems.

High levels of cortisol are associated with childhood trauma, and disruption of normal functioning, with negative effects on the developing brain. Childhood trauma appears to have long-term impacts on gene activity without changing the sequence of an individual's DNA, through epigenetic changes.⁹ Childhood trauma can result in epigenetic modifications of genes related to the hypothalamic-pituitary-adrenal (HPA) and hypothalamic-pituitary-gonadal axes and the immune system, creating downstream psychological and physical problems, including modifications in how the individual responds to punishment specifically and stress more generally. Sexual abuse is uniquely associated with greater likelihood of serious psychological symptoms, and greater symptom severity, compared to other types of trauma. Mark's experiences of repeated trauma, including sexual, physical, and psychological abuse, in addition to neglect and severe attachment disruption, very likely had a profound influence on how his body and brain developed over time. Specifically, **his trauma history likely delayed brain maturation processes as well as contributed to his cognitive impairments and emotional problems at the time of the offense.**

Mark Is at Low Risk of Future Violence

I routinely conduct violence risk assessments of individuals who committed murders during adolescence or early adulthood, for the purpose of juvenile adjudications and sometimes resentencings. **Mark's current presentation and his history are consistent with a relatively low risk of future violence, even in light of the facts of the instant offense and the severe and chronic trauma of his childhood.** He has matured, developmentally, and shows markedly less impulsivity and

⁸ Carrion VG, et al. (2010). Reduced hippocampal activity in youth with posttraumatic stress symptoms: an fMRI study. *Journal of Pediatric Psychology*, 35(5), 559-569.

⁹ DeBellis MD, Zisk A (2014). The biological effects of childhood trauma. *Child and Adolescent Clinics of North America* 23(2), 185-222.

instability than he did as an adolescent. His temperament and personality style in adulthood, as evidenced by his adjustment to prison, work performance, and observations of correctional staff, are prosocial and non-aggressive. He has shown overall excellent rule adherence, and he interacts with the staff and correctional officers similarly to the way that a dependent child interacts with caregivers. One of the most striking aspects of Mark's presentation is the **absence of dysregulated anger and bitterness, and the presence of prosocial instead of antisocial personal values, despite having a severe trauma history starting in infancy**, with essentially no relief until he entered prison. In prison, he has been surrounded by other incarcerated individuals, most of whom committed very serious and violent crimes, for decades. Mark's adaptation to his ID and trauma symptoms has included a highly agreeable (i.e., appeasing), interpersonally submissive, and compliant attitude. He tries to model for others how he hopes to be treated himself, and he has tried to construct healthy and positive relationships with people whose values he admires and whose connections he values, like Shari Seale and Vickie and Charles Blanton.

My findings with regard to violence risk in Mark were consistent with Dr. Sorensen's findings in his formal risk assessment calculations.

DIAGNOSTIC CONCLUSIONS

I am in agreement with Dr. Ouaou's diagnosis of **Intellectual Disability**, which is a type of Developmental Disability. Given the historical context of Mark's childhood education, it is fair to say that a child with his collection of impairments would be assessed and treated very differently today in an educational setting.

Mark also meets criteria for **Post Traumatic Stress Disorder (PTSD)**. More specifically, he exhibits Developmental Trauma/Disorders of Extreme Stress Not Otherwise Specified (DESNOS), sometimes referred to as **Complex Post Traumatic Stress Disorder (C-PTSD)**. C-PTSD refers to the broad and pervasive effect of experiences of abuse and neglect, particularly repeated abuse and neglect, as well as lack of stable and loving caregiver relationships, on children and adolescents. This type of interpersonal trauma, happening over and over, without any protective adult relationships, has different effects on a person compared to a single traumatic stressor event in an otherwise healthy adult. The two key features that differentiate developmental trauma (DESNOS and C-PTSD) from standard PTSD are (a) long-term changes in how the brain and body develop in the traumatized individual, and (b) broad and profound effects on how the individual perceives the self and relationships.

Symptoms commonly seen in C-PTSD/DESNOS include suicidality and self-harming behavior, excessive risk taking, uncontrolled anger, episodes of dissociation and/or feelings of depersonalization, preoccupation with imagined or real abandonment, victimizing others, digestive system problems, chronic pain without identified medical cause, sense of foreshortened future, and

cognitive problems such as inattentiveness and memory impairment. Mark has exhibited some but not all of these symptoms, and in my experience anger is very frequently one of the major problems for individuals with C-PTSD. He has utilized other adaptations instead of anger, namely a high degree of interpersonal agreeableness, a tendency to behave in a submissive and appeasing manner toward others, and efforts to make himself useful by helping others with practical tasks when he can. At the time of the offense in 1989, however, Mark was still in the midst of adolescent development, he had an unstable lifestyle and severe trauma symptoms, was not connected to prosocial influences, and he was using drugs and alcohol excessively. It does not surprise me that dysregulated anger came to the fore when he was highly intoxicated, unstable, and interacting with an age peer at the age of 21, and in fact it is remarkable that he did not exhibit more aggression in the years prior to the instant offense, given his history.

Mark has been treated for depressive-type symptoms in the past, but his depression is better understood as a facet of his PTSD rather than a freestanding separate mood disorder. He has no serious mental illness or personality problems that would contribute to future violence risk. Instead, he is at very low risk of perpetrating violence at this point in his life, and he is at **low risk of engaging in serious violent acts in prison, even if he is moved off of death row into general population**. My findings, and the findings of Dr. Sorensen, a prison violence risk expert, were congruent; as Dr. Sorensen stated, "The empirical analysis also demonstrates that Mr. Jenkins would have a very low likelihood of committing serious or violent rule infractions relative to the general prison population" if he were held in less restrictive prison conditions.

Respectfully submitted,

A handwritten signature in black ink, appearing to read 'SBE', with a long horizontal flourish extending to the right.

Sara E. Boyd, Ph.D., ABPP
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Board Certified Forensic Psychologist
American Board of Professional Psychology